



ROOT CAUSE UNLOCKED

The Complete Patient's Guide to *Sciatica*

Natural Strategies for Nerve Pain Relief, Healing, and Recovery

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BEFORE YOU START

Disclaimer: This ebook is for educational purposes only and does not constitute medical advice. Always consult with a qualified healthcare provider before starting any new treatment, supplement, exercise program, or dietary change. Do not start, stop, or change any prescribed medication without talking to the provider who prescribed it. If you experience progressive weakness, loss of bowel or bladder control, or severe worsening pain, seek emergency medical attention immediately.

A note on the evidence in this guide. Every claim below that carries a numbered marker has been checked against the actual published study. Where a statement is general clinical background rather than a specific research finding, it carries no marker. Claims that could not be traced to a real, on-point study were removed rather than softened.

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Working with Root Health

1. What Is Sciatica?

Sciatica refers to pain that radiates along the path of the sciatic nerve, the largest and longest nerve in your body. This nerve branches from your lower back through your hips and buttocks and down each leg. Typically, sciatica affects only one side of the body.^{1,2}

Contrary to popular belief, sciatica is not a condition itself but rather a symptom of an underlying problem affecting the sciatic nerve or its nerve roots. Understanding this distinction matters, because effective treatment means addressing the source of the irritation, not just masking the pain.²

Common Symptoms

Common symptoms include sharp, burning, or shooting pain that travels from the lower back down the leg, and numbness, tingling, or “pins and needles” in the leg or foot. Muscle weakness in the affected leg can occur, and pain often worsens with sitting, coughing, sneezing, or straining. Some people also notice difficulty moving the leg or foot.

2. Understanding the Anatomy

The sciatic nerve is formed by the convergence of nerve roots from the L4, L5, S1, S2, and S3 spinal segments. It exits the pelvis through the greater sciatic foramen and travels down the posterior thigh, branching into smaller nerves that serve the lower leg and foot.

Because the nerve is so long, irritation can occur at multiple points along its path. At the spinal level, herniated discs, spinal stenosis, or spondylolisthesis can compress a nerve root. At the pelvic level, piriformis syndrome occurs when the piriformis muscle compresses the nerve. And at the hamstring level, entrapment can occur from fibrous bands or hamstring tendons.

Understanding where your symptoms originate helps guide the most effective treatment approach.

3. Common Causes and Risk Factors

Primary Causes

Lumbar Herniated Disc (the most common cause): the gel-like center of an intervertebral disc pushes through the tough outer layer, pressing on a nerve root. Lumbar disc herniation is the most frequently identified structural cause of sciatica, although published estimates of how often it is responsible vary widely between studies and definitions.^{1,2}

Degenerative Disc Disease: as discs age, they lose height and water content, reducing the space where nerves exit the spine.

Lumbar Spinal Stenosis: narrowing of the spinal canal, often age-related, compresses the nerve roots.

Spondylolisthesis: one vertebra slips forward over another, narrowing the nerve exit space.

Piriformis Syndrome: the piriformis muscle, deep in the buttock, spasms or tightens and compresses the sciatic nerve. This is a recognized but less common source of sciatic-type pain.

Risk Factors

Several factors increase the risk of developing sciatica. Age is one, since it is most common between 30 and 50 years.¹ Prolonged sitting or a sedentary lifestyle, and occupations requiring heavy lifting or twisting, both add mechanical stress. Obesity increases spinal load and is an established risk factor for sciatica.^{46,47} Diabetes increases nerve damage risk, smoking is associated with reduced blood flow to spinal discs, and pregnancy carries its own risk through hormonal changes and shifts in weight distribution.

4. How Sciatica Is Diagnosed

A proper diagnosis typically involves:

Clinical Examination

A clinical examination typically includes a straight leg raise test (Lasègue's sign), a neurological examination of strength, reflexes, and sensation, gait analysis, and palpation of the spine and pelvis.

Imaging (when needed)

When imaging is needed, MRI is the preferred study for visualizing soft tissues, discs, and nerve compression. CT scanning is useful for bony detail, while X-rays have limited utility for soft tissue but are helpful for ruling out fractures or alignment issues. EMG and nerve conduction studies assess nerve function when the diagnosis is unclear.

Important: many people have MRI findings such as disc bulges without any symptoms, and clinical guidelines advise against routine imaging for back pain in the absence of red flags or a plan for surgery. Imaging should correlate with your clinical presentation, not drive treatment decisions on its own.³

5. The Healing Timeline: What to Expect

Understanding the natural history of sciatica helps set realistic expectations.

Acute phase (0 to 6 weeks): pain is usually most intense. A large share of cases improve substantially during this window with conservative care alone. Prospective follow-up of sciatica associated with disc pathology has shown that symptoms and the disc herniation itself often resolve over time without surgery.⁴

Subacute phase (6 to 12 weeks): gradual improvement continues. This is when targeted rehabilitation becomes most important.

Chronic phase (beyond 12 weeks): if pain persists, a more comprehensive, multidisciplinary approach is warranted.

Key insight. Randomized evidence comparing early surgery with prolonged conservative care for sciatica from a lumbar disc herniation found that early surgery gave faster relief of leg pain, but that outcomes at one year were similar between the two groups.⁶ The SPORT randomized trial and a systematic review of surgery versus conservative management reached a compatible conclusion: surgery tends to speed up recovery, while long-term outcomes converge.^{5,7} Ten-year follow-up from the Maine Lumbar Spine Study likewise found that patients treated surgically and nonsurgically reported broadly similar long-term outcomes on several measures.⁸

This is why an unhurried, well-structured conservative plan is a reasonable first step for most people who do not have red-flag findings.

6. Evidence-Based Supplements for Nerve Health and Inflammation

The supplements below have research support of varying strength. Read the scope of each finding carefully. Much of the strongest supplement research in this area comes from joint pain or diabetic nerve pain, not from sciatica specifically, and that distinction is stated wherever it applies.

Always discuss supplements with your healthcare provider, especially if you take medications, since nothing here is a cure or a substitute for prescribed treatment.

6.1 Omega-3 Fatty Acids (EPA and DHA)

What it does: EPA and DHA are incorporated into cell membranes and serve as precursors to resolvins and protectins, a family of mediators involved in actively resolving inflammation rather than only suppressing it. Marine omega-3 fatty acids influence multiple steps in the inflammatory process.⁹

What the evidence shows: the mechanistic and biochemical case for omega-3 fatty acids in inflammation is well described.⁹ There is no high-quality trial evidence showing that omega-3 supplementation relieves sciatica, so treat this as general anti-inflammatory support rather than a sciatica treatment.

Typical intake: 1,200 to 2,400 mg combined EPA and DHA daily. That is the range used in the one study run in people with nonsurgical neck and back pain, where most participants took 1,200 mg daily and about a fifth took 2,400 mg. That study was an uncontrolled questionnaire follow-up rather than a placebo-controlled trial, so treat it as a starting point rather than a proven dose. Higher intakes appear in the joint-pain literature, but that work was done in rheumatoid arthritis and other inflammatory conditions rather than in the mechanical and degenerative problems this guide covers, so I have not carried its dose across.⁵³ **Form:** triglyceride or re-esterified triglyceride forms are commonly chosen for absorption. **Caution:** may increase bleeding risk at higher doses. Tell your surgeon if a procedure is planned.

6.2 Turmeric and Curcumin

What it does: curcumin, the active compound in turmeric, acts on NF-κB and on inflammatory enzymes including COX-2.

What the evidence shows: a systematic review and meta-analysis of randomized clinical trials found that turmeric extracts and curcumin reduced arthritis symptoms, while noting that the number and size of the trials were limited.¹⁰ A multicenter randomized trial in knee osteoarthritis found a *Curcuma domestica* extract comparable to ibuprofen for pain and function over four weeks.¹¹ Both trials were conducted in joint arthritis populations; neither one addresses sciatica or nerve pain.

Typical intake: 500 to 1,000 mg curcuminoids daily, often taken with piperine (black pepper extract) or in a liposomal or phytosome form for absorption. **Caution:** may interact with blood thinners. Discuss before use if you have gallbladder disease.

6.3 Vitamin D

What it does: supports bone health, muscle function, and immune regulation.

What the evidence shows: a study of patients with persistent, nonspecific musculoskeletal pain found a high prevalence of severe vitamin D deficiency in that group.¹² This shows how common deficiency was in that group; it does not demonstrate that correcting it relieves pain. A systematic review of vitamin D supplementation in diabetic peripheral neuropathy reported improvement in neuropathic pain scores, while flagging the low quality of the available studies.¹³ Checking your level is reasonable. Expecting vitamin D alone to resolve sciatica is not.

Typical intake: dosing should be guided by a blood level. Many clinicians target a level in the sufficient range and dose accordingly. **Form:** D3 (cholecalciferol). **Caution:** have levels checked periodically. Toxicity is uncommon but possible at very high intakes.

6.4 Magnesium

What it does: magnesium acts as an NMDA receptor antagonist, a mechanism relevant to central sensitization and pain amplification, and it participates in normal muscle and nerve function.

What the evidence shows: a review of magnesium and pain summarizes the NMDA-related mechanism and the mixed clinical trial results across pain conditions.¹⁴ No trial evidence addresses sciatica specifically.

Typical intake: 200 to 400 mg elemental magnesium daily. Glycinate and threonate are commonly chosen for tolerability; citrate is often used when constipation is also an issue. This is a general supplemental intake range rather than a dose drawn from a trial in this condition, which is why no study is cited beside it. **Caution:** can cause loose stools. Avoid in severe kidney disease unless your physician directs otherwise.

6.5 Methylcobalamin (Active Vitamin B12)

What it does: vitamin B12 is required for myelin maintenance, the protective sheath around nerves.

What the evidence shows: a systematic review of clinical controlled trials of vitamin B12 in diabetic neuropathy found that supplementation improved some symptom measures, but concluded the evidence base was limited and of uneven quality.¹⁵ The trials reviewed were in diabetic neuropathy; none examined sciatica.

Typical intake: 1,000 to 2,000 mcg daily, sublingual or oral. **Caution:** generally very well tolerated.

6.6 Alpha-Lipoic Acid (ALA)

What it does: an antioxidant studied extensively in diabetic nerve damage, where oxidative stress is part of the mechanism.

What the evidence shows: the SYDNEY trial found that intravenous alpha-lipoic acid improved sensory symptoms in diabetic polyneuropathy.¹⁶ The follow-up SYDNEY 2 trial found that oral alpha-lipoic acid at 600 mg daily improved symptomatic diabetic polyneuropathy, with higher doses adding side effects rather than benefit.¹⁷ A meta-analysis of alpha-lipoic acid in symptomatic diabetic polyneuropathy supported short-term symptom improvement.¹⁸ Every one of these trials was conducted in diabetic polyneuropathy; sciatica was not studied.

Typical intake: 600 mg daily is the dose used in the oral trial. **Caution:** may lower blood sugar. If you take diabetes medication, coordinate with your prescriber before adding it.

6.7 Palmitoylethanolamide (PEA)

What it does: a naturally occurring fatty acid amide that acts on mast cells and glial cells, pathways involved in neuroinflammation.

What the evidence shows: a pooled data meta-analysis reported that PEA reduced pain intensity across chronic pain conditions with a favorable tolerability profile.¹⁹ A later systematic review and meta-analysis of double-blind randomized controlled trials also found a reduction in pain, while noting heterogeneity across studies.²⁰ Little of this trial evidence was gathered in sciatica specifically.

Typical intake: 300 to 600 mg twice daily, micronized form commonly used. The upper end of that range is the dose used in a randomized trial of ultra-micronized PEA at 600 mg twice daily in older adults with chronic pain.⁵⁰ **Caution:** generally well tolerated with few reported side effects.

6.8 Boswellia Serrata (Frankincense)

What it does: inhibits 5-lipoxygenase, an enzyme in the inflammatory cascade, reducing leukotriene production.

What the evidence shows: a systematic review and meta-analysis found Boswellia and Boswellia extract effective for pain, stiffness, and function in osteoarthritis patients.²¹ The population studied was osteoarthritis, so this does not establish a benefit for nerve pain.

Typical intake: 300 to 500 mg standardized extract twice daily. **Caution:** discuss with your provider if you take anti-inflammatory medications.

6.9 Ginger

What it does: contains gingerols and shogaols, compounds with anti-inflammatory activity.

What the evidence shows: a randomized study found that daily ginger supplementation reduced muscle pain caused by eccentric exercise.²² A systematic review of ginger as an analgesic and ergogenic aid in sport found modest analgesic effects and limited, inconsistent performance effects.²³ None of this is nerve pain research, and no ginger trial has tested a sciatica claim.

Typical intake: 500 to 1,000 mg standardized extract daily, or fresh ginger in cooking. The randomized trial of a standardized ginger extract in knee osteoarthritis dosed it twice daily; the milligram amount is not stated in its abstract.⁵² **Caution:** may interact with blood thinners and can cause stomach upset at higher doses.

6.10 Collagen (Type II)

What it does: provides amino acid building blocks associated with cartilage and connective tissue.

What the evidence shows: a clinical trial of undenatured type II collagen in knee osteoarthritis reported reduced joint pain compared with a glucosamine and chondroitin combination.²⁴ The finding is specific to the knee joint; no evidence shows collagen supplementation repairs intervertebral discs or relieves sciatica.

Typical intake: 40 mg undenatured type II collagen daily, or 10 g hydrolyzed collagen as a general connective tissue support. The 40 mg figure is the exact daily dose used in the multicenter randomized trial of undenatured type II collagen in knee osteoarthritis.⁵¹ **Caution:** generally very safe. Source quality varies.

7. Targeted Exercises for Sciatica Relief

Critical rule: stop any exercise that increases radiating leg pain, numbness, or weakness. Mild discomfort in the back or buttock is usually acceptable, but pain traveling farther down the leg indicates nerve irritation and is a signal to back off.

Exercise as a category has broad support in chronic low back pain and chronic pain generally, with modest effect sizes and a good safety profile.^{28,30}

Before You Begin: The Pre-Treatment Warm-Up

Best for: Everyone, before any of the exercises below. This is soft-tissue release, not a substitute for the nerve-specific work that follows, and it is the same sequence used in office at Root Health, tools and doses unchanged. Use a percussion massage device, a foam roller, or your own hands, whichever you have, and respect a 6 out of 10 pressure ceiling throughout. The same critical rule from above applies here too: stop if anything increases radiating leg pain, numbness, or weakness.

Cat-Camel: 1. On hands and knees, arch your back up, then let it sag. 2. 1 set of 10 repetitions.

Glute Release, All Three Muscles: 1. With a percussion device, cover the glute maximus, medius, and minimus. 2. 1 to 2 minutes per side.

TFL and Hip Flexor Release: 1. With the same device, run it over the TFL and hip flexors. 2. 1 to 2 minutes each side.

Deep Hip Rotator and IT Band Release: 1. Lying on your back with a massage ball or foam roller under the glutes, move the leg up and down, or sit on the roller and search for tender spots. 2 to 3 minutes per side. 2. Side-lying, roll the outer thigh from just below the hip to above the knee for the IT band. 1 to 2 minutes per side.

This is the region worth lingering on if your pain sits mostly in the buttock: the piriformis and the other deep rotators run directly alongside the sciatic nerve, and section 7.3 below gives you the stretch that follows naturally from this release work.

Psoas Release: 1. With a massage ball or a lacrosse ball, place it over the front of the abdomen where the psoas runs. 2. Rock gently back and forth, then up and down. 3. 20 to 30 seconds each side.

This one is unusual and it is deliberate: the psoas connects the lumbar spine to the leg, and it rarely gets touched by anything else.

Why it is here: this is preparation, not treatment. It is what gets the tissue loose enough that the exercises below, which are the actual evidence-backed part of this chapter, are tolerable to do. If a step here increases leg symptoms rather than easing them, skip it and move on to the exercises.

7.1 McKenzie Extension Exercises

Best for: disc-related sciatica, especially when pain worsens with sitting and bending and improves with standing and walking.

Prone on Elbows 1. Lie face down with elbows under your shoulders. 2. Relax your lower back and hips completely. 3. Hold for 2 to 3 minutes, breathing deeply. 4. Repeat 3 to 5 times daily.

Press-Up (Prone Press-Up) 1. Lie face down, hands under shoulders as if doing a push-up. 2. Keep your hips and pelvis in contact with the floor. 3. Slowly press your upper body up, extending your arms. 4. Hold at the top for 2 seconds, then lower. 5. Start with 5 repetitions and progress to 10. 6. During an acute flare, many clinicians have patients repeat this at intervals through the day.

Why it works: the McKenzie approach, formally called Mechanical Diagnosis and Therapy, classifies patients by their response to repeated end-range movements and prescribes the direction that reduces symptoms. A literature review with meta-analysis found the McKenzie method more effective than comparison interventions for pain and disability in low back pain, with the authors noting that the effect sizes were small and the evidence quality limited.²⁵

7.2 Nerve Glides (Neural Flossing)

Best for: restricted nerve mobility anywhere along the pathway.

Sciatic Nerve Glide (Supine) 1. Lie on your back, legs straight. 2. Bring the affected leg up toward the ceiling, holding behind the thigh. 3. Slowly flex your ankle (pull toes toward you) while extending the knee. 4. Then point the toes while slightly bending the knee. 5. This creates a gliding motion along the nerve. 6. Perform 10 slow, controlled repetitions. 7. Never push into pain. A mild pulling or tingling is acceptable, sharp pain is not.

Why it works: neural mobilization aims to restore normal gliding of the nerve relative to surrounding tissue. A systematic review of neural mobilization in low back and radicular pain found evidence of benefit for pain and disability, with the authors cautioning that study quality varied.²⁶

7.3 Piriformis Stretching

Best for: piriformis-related symptoms, or when pain is primarily in the buttock.

Supine Piriformis Stretch (Figure 4) 1. Lie on your back, knees bent, feet flat. 2. Cross the affected leg over the opposite knee, creating a figure 4. 3. Gently pull the uncrossed thigh toward your chest. 4. You should feel a stretch in the buttock of the crossed leg. 5. Hold for 30 seconds, repeat 3 times.

Seated Piriformis Stretch 1. Sit in a chair and cross the affected leg over the opposite knee. 2. Keep your back straight and gently lean forward. 3. Hold for 30 seconds, repeat 3 times.

Why it works: the piriformis lies close to the sciatic nerve. When it is tight or in spasm it can contribute to buttock and posterior leg symptoms. Stretching aims to reduce that tension.

7.4 Core Stabilization

Best for: long-term support and prevention.

Dead Bug 1. Lie on your back, arms extended toward the ceiling, knees bent at 90 degrees. 2. Slowly lower one arm behind you while extending the opposite leg. 3. Keep your lower back pressed firmly into the floor. 4. Return to start and alternate sides. 5. Perform 8 to 10 reps per side, 2 to 3 sets.

Bird Dog 1. Start on hands and knees in a tabletop position. 2. Extend one arm forward and the opposite leg back. 3. Keep your core engaged and your back flat. 4. Hold for 5 seconds, then return to start. 5. Perform 8 to 10 reps per side, 2 to 3 sets.

Why it works: spinal stability comes from coordinated activity of many trunk muscles rather than from any single muscle, and stability demands change with the task.²⁷ In practice, exercise programs for chronic low back pain produce modest improvements in pain and function, and no single exercise type has been shown to be clearly superior to the others.²⁸

7.5 Hamstring and Hip Flexor Stretching

Best for: reducing tension across the lumbar spine and pelvis.

Standing Hamstring Stretch 1. Place the affected leg on a low step or chair. 2. Keep the knee slightly bent, not locked. 3. Hinge forward at the hips, keeping your back straight. 4. Hold for 30 seconds, repeat 3 times.

Half-Kneeling Hip Flexor Stretch 1. Kneel on one knee, other foot forward in a lunge position. 2. Tuck your pelvis under (posterior tilt). 3. Gently shift your weight forward. 4. Hold for 30 seconds, repeat 3 times.

Why it works: tight hamstrings and hip flexors change pelvic mechanics and can add stress to the lumbar spine. Note that in sciatica, a hamstring stretch can also tension the nerve itself, so ease into it and stop if leg symptoms increase.

7.6 Walking

Best for: almost everyone. It is foundational.

Protocol

Start with 10 to 15 minutes at a comfortable pace, then gradually build toward 30 to 45 minutes daily as tolerated. Maintain an upright posture, avoid hills initially if they aggravate symptoms, and stick to flat, even surfaces during acute phases.

Why it works: walking maintains mobility and load tolerance without high spinal stress. A systematic review and meta-analysis comparing walking with other exercise in chronic low back pain found no significant difference between them for pain and disability, which means walking is a reasonable, low-barrier option.²⁹

8. Physical Therapies That Work

8.1 Physical Therapy

Physical therapy is a cornerstone of conservative sciatica management. A skilled physical therapist will assess your specific movement patterns and identify aggravating factors, prescribe individualized exercises based on your presentation, use manual therapy techniques to improve joint mobility, educate you on body mechanics and posture, and progress you through a structured rehabilitation program.

Evidence: a systematic review of conservative management for lumbar disc herniation with associated radiculopathy found support for advice to stay active and for structured exercise programs, while noting that no single conservative protocol has been shown to be clearly superior.³¹ The American College of Physicians guideline recommends nonpharmacologic treatment, including exercise, as first-line care for low back pain.³²

8.2 Chiropractic Care and Spinal Manipulation

Spinal manipulation involves controlled, specific forces applied to spinal joints with the aim of improving mobility and reducing symptoms.

Evidence: a systematic review and meta-analysis found that spinal manipulative therapy was associated with modest improvements in pain and function in acute low back pain, with transient minor musculoskeletal soreness the most common adverse effect.³³ The American College of Physicians guideline includes spinal manipulation among the nonpharmacologic options for acute and subacute low back pain.³² A network meta-analysis of management strategies specifically for sciatica compared a wide range of options and found the evidence base uneven, with no clear ranking of conservative treatments against one another.³⁴

Be aware that most of the manipulation evidence above is for low back pain, and that sciatica with true nerve root involvement is a distinct and less-studied population.

What to expect: care typically combines manipulation or mobilization with exercise prescription and activity advice. Ask your provider for a defined plan of care with a scheduled re-evaluation point, so that progress is measured rather than assumed.

8.3 Acupuncture

Acupuncture involves inserting thin needles at specific points with the aim of modulating pain processing.

Evidence: an updated individual patient data meta-analysis found acupuncture superior to both sham acupuncture and no acupuncture for chronic musculoskeletal pain, with effects that persisted over time, although the difference from sham was modest.³⁵ A systematic review and meta-analysis focused on sciatica found that acupuncture improved pain outcomes compared with control interventions, with the authors noting that the included trials were of limited methodological quality.³⁶

What to expect: most protocols involve one to two sessions per week over several weeks. Benefits tend to accumulate across sessions.

8.4 Massage Therapy and Myofascial Release

Therapeutic massage addresses muscle tension and trigger points that may contribute to symptoms.

Evidence: a randomized controlled trial comparing structural massage, relaxation massage, and usual care for chronic low back pain found that both massage types produced better function and reduced symptoms than usual care, with benefits largely persisting at six months.³⁷ That trial studied chronic low back pain, not sciatica specifically.

Focus areas

Focus areas typically include the gluteal muscles, including the piriformis and gluteus medius and minimus, the lumbar paraspinals, the hamstrings, and the hip flexors.

8.5 Heat and Cold Therapy

Cold therapy (often used in the first 48 to 72 hours of an acute flare)

Cold therapy may help numb pain in the acute period. Apply an ice pack wrapped in cloth for 15 to 20 minutes, and allow the skin to return to normal temperature between applications.

Heat therapy (after the acute phase, or for muscle spasm)

Heat increases local blood flow and can reduce the sensation of stiffness. Use a heating pad or warm bath for 15 to 20 minutes; many people find it useful before stretching or exercise.

Evidence: a Cochrane review of superficial heat or cold for low back pain found moderate evidence that heat wrap therapy reduces pain and disability in acute and subacute low back pain, and found insufficient evidence to evaluate cold therapy.³⁸ In other words, heat has better support than ice, and neither has been shown to change the course of sciatica.

8.6 TENS (Transcutaneous Electrical Nerve Stimulation)

TENS units deliver low-voltage electrical currents through the skin with the aim of modulating pain signals.

Evidence: a Cochrane review of TENS for acute pain concluded that the available evidence was inconclusive because of the small number and low quality of eligible trials.³⁹ TENS is generally safe and can be tried at home, but you should treat it as a comfort measure with uncertain evidence rather than an established treatment.

Usage: place electrodes along the pain pathway. Sessions of 20 to 30 minutes are typical. Follow the device manufacturer's instructions and your provider's guidance.

8.7 Epidural Steroid Injections

For severe, persistent sciatica, an epidural injection delivers medication near the inflamed nerve root.

Evidence: a systematic review of randomized controlled trials comparing saline, local anesthetics, and steroids in epidural and facet joint injections found that epidural injections can provide meaningful relief in spinal pain, and notably that the injectate itself, including local anesthetic alone, often accounted for a substantial part of the effect.⁴⁰ A separate randomized trial in lumbar spinal stenosis, a different condition from disc-related sciatica, found that adding a glucocorticoid to lidocaine offered minimal or no additional benefit at six weeks.⁴¹

Considerations: injections are usually limited in number per year because of potential side effects. Discuss the risks, including effects on blood sugar and bone density, with your physician. Injections are best understood as a possible bridge that makes rehabilitation feasible, not as a stand-alone solution.

9. The Anti-Inflammatory Diet for Nerve Recovery

What you eat influences overall health and inflammatory load. The dietary pattern below is a sensible, well-tolerated way of eating. Be clear-eyed about what it is: general health support, not a proven sciatica treatment.

9.1 Foods to Emphasize

Emphasize omega-3 rich foods such as wild-caught salmon, sardines, mackerel, and anchovies, along with walnuts, flaxseeds, and chia seeds, aiming for 2 to 3 servings of fatty fish per week. Fill out your plate with colorful vegetables and fruits: leafy greens such as spinach, kale, and Swiss chard, berries such as blueberries, strawberries, and blackberries, cruciferous vegetables such as broccoli, cauliflower, and Brussels sprouts, and orange and yellow vegetables such as carrots, sweet potatoes, and peppers, with the goal of a large and varied intake spread across the day. Anti-inflammatory spices such as turmeric (paired with black pepper), ginger, garlic, cinnamon, and rosemary are worth incorporating regularly, along with healthy fats such as extra virgin olive oil, avocados, and nuts and seeds. Quality protein sources include fish, poultry, meat, eggs, and legumes as tolerated, and bone broth and other collagen sources provide a traditional source of glycine and proline.

9.2 Foods to Minimize

Minimize pro-inflammatory fats such as trans fats (partially hydrogenated oils), deep-fried foods, and very high intakes of refined seed oils relative to omega-3 sources. Cut back on refined carbohydrates and added sugars, including white bread, pastries, and sugary cereals, soda and sweetened beverages, and candy and desserts. Highly processed foods such as deli meats, hot dogs, bacon, fast food, and packaged snacks are also worth minimizing. Excessive alcohol interferes with sleep, which matters for recovery, and heavy intake is associated with poorer nutritional status. Some individuals notice a pattern with nightshades such as tomatoes, potatoes, eggplant, and peppers, though most people are not sensitive to them; this is worth testing individually rather than assuming.

9.3 Hydration

Intervertebral discs are highly hydrated tissues that rely on diffusion for nutrition, so staying well hydrated is sensible.

A practical rule of thumb: drink enough that you are rarely thirsty and your urine stays pale through the day. Individual needs vary with body size, climate, and activity.

Tips

Start your day with a glass or two of water, and drink before meals. Be aware that caffeine and alcohol increase fluid losses, and add electrolytes if you sweat heavily.

9.4 Sample Anti-Inflammatory Day

A sample anti-inflammatory day might begin with a smoothie of spinach, blueberries, flaxseed, ginger, almond butter, and unsweetened almond milk, or oatmeal with walnuts, chia seeds, cinnamon, and berries. Lunch could be a large salad with mixed greens, grilled salmon, avocado, and an olive oil and lemon dressing, with a side of vegetable soup. An afternoon snack of a handful of walnuts and an apple, or carrot and celery sticks with hummus, helps keep energy steady. Dinner might feature grilled chicken or tofu with turmeric and garlic, roasted broccoli and sweet potato, and a small side salad with olive oil. In the evening, if you like, a cup of ginger or turmeric tea and a small square of dark chocolate round out the day.

10. Lifestyle Modifications and Pain Management

10.1 Sleep Optimization

Sleep and pain influence each other in both directions. Poor sleep tends to worsen pain, and pain disrupts sleep.⁴³

Strategies

Maintain a consistent sleep schedule, with the same bedtime and wake time, and sleep on your side with a pillow between your knees, or on your back with a pillow under your knees. Many people find stomach sleeping aggravating for the low back. Keep your room cool, limit screens in the hour before bed, and consider that some people find magnesium glycinate in the evening helpful (see section 6.4).

Evidence: an experimental neuroimaging study found that a night of sleep deprivation lowered pain thresholds and altered pain-related activity in the brain, including heightened somatosensory cortex reactivity and blunted activity in regions involved in pain modulation.⁴² A review of the sleep and pain relationship concluded that sleep disturbance is a strong predictor of pain, arguably more so than the reverse.⁴³

10.2 Stress Management

Chronic stress affects pain perception and coping capacity.

Effective techniques

Effective techniques include diaphragmatic breathing for 5 minutes, twice daily, and mindfulness meditation: a randomized clinical trial found that mindfulness-based stress reduction improved back pain and functional limitations compared with usual care, with results similar to cognitive behavioral therapy.⁴⁴ Progressive muscle relaxation, systematically tensing and releasing muscle groups, is another option, as is gentle yoga; a randomized noninferiority trial found yoga noninferior to physical therapy for pain and function in chronic low back pain.⁴⁵

Note that both of those trials studied chronic low back pain rather than sciatica specifically.

10.3 Ergonomics and Body Mechanics

Sitting

Use a chair with lumbar support, or add a lumbar roll, keep your feet flat on the floor or on a footrest, and position your hips slightly higher than your knees. Take standing or walking breaks every 30 to 45 minutes, and consider a sit-stand desk.

Standing

Shift your weight periodically, use an anti-fatigue mat, and rest one foot on a small step to reduce lumbar extension.

Lifting

Bend at the knees and hips, not the waist, and keep the load close to your body. Avoid twisting while lifting, and ask for help with heavy items.

Driving

Adjust the seat so your knees are slightly bent, use a lumbar support, and take breaks every 1 to 2 hours on long trips.

10.4 Weight Management

Evidence: a meta-analysis found that overweight and obesity are associated with an increased prevalence and incidence of low back pain.⁴⁶ A separate meta-analysis found that obesity is a risk factor for sciatica specifically.⁴⁷ These are association findings. A systematic review of weight loss programs for low back pain found the evidence that intentional weight loss reduces back pain to be limited and inconsistent.⁴⁸

The honest summary: carrying less weight reduces spinal load and is good for your general health, but do not expect weight loss on its own to resolve sciatica.

Approach

Focus on the dietary pattern outlined above, aim for gradual and sustainable change, and combine it with walking and gentle exercise as tolerated.

10.5 Smoking Cessation

Smoking is associated with poorer spinal health and reduced blood supply to spinal tissues, and it is associated with worse outcomes across many musculoskeletal conditions. If you smoke, stopping is one of the highest-value changes you can make for your overall health. Ask your provider about cessation support.

11. The In-Clinic Pre-Treatment Protocol

Understand what this is for, because the distinction matters.

None of this rebuilds tissue. The loading and strengthening work in the previous chapters does that. What this protocol does is reduce the muscle tone, stiffness and tissue sensitivity that make the strength work harder to tolerate and less pleasant to do, particularly in the early weeks when things are at their most irritable.

It is preparation, not treatment. I use it in the clinic before the working part of a session, and I give it to patients to use at home before theirs. Patients who do it tend to tolerate the loading better and stick with it longer, and sticking with it is the actual mechanism by which recovery happens.

If you have to choose between doing this and doing your strength work, do your strength work.

Heat and vibration back wrap, set to vibrate and heat, while performing:

- Cat-camel stretch, 1 set of 10.
- Hamstring stretch with a stretch strap, 1 set of 10 each side.
- Ball bounce on an exercise ball, up and down then side to side, 1 set of 10 each, palms up.

Vibrating massage ball or foam roller. Lie on your back with the sphere on the glute muscles, moving the leg up and down.

Percussion massager, round or flat head. Over the tensor fasciae latae and hip flexors, 1 to 2 minutes each side.

Massage ball over the abdomen, working the psoas, rocking back and forth and up and down, 20 to 30 seconds each side.

Posture Curve under the lower back, knees bent to start and straightening if comfortable, resting 30 to 60 seconds.

Vibrating sphere or foam roller on sore calf areas, moving the foot up, down, left, right and in circles, 2 to 3 reps per area.

Safety, and I mean these.

- Avoid all of this over an acutely injured area in the first 48 to 72 hours.
- Reduce pressure over anything actively inflamed.
- Skip percussion therapy entirely if you have a pacemaker or another implanted electronic device.
- Avoid aggressive pressure over varicose veins or compromised skin.

- Start light. Pain must not exceed 6 out of 10. Back off immediately for sharp or shooting pain.
- Stop and seek advice if you get numbness, tingling, or weakness.

One absolute rule before you roll or percuss a calf. Never work on a calf that is swollen, warm, or tight in one leg only, particularly after travel, surgery, or a period of immobilisation. That is a blood clot concern. It needs assessing the same day and it must never be massaged.

12. Red Flags: When to Seek Immediate Care

While most sciatica improves with conservative care, certain symptoms require urgent medical evaluation.^{2,3}

Seek immediate care if you experience:

- **Cauda equina syndrome signs:** loss of bowel or bladder control, urinary retention or new difficulty urinating, or numbness in the saddle area (inner thighs, genitals, buttocks). **This is a surgical emergency. Go to an emergency department immediately, do not wait for an office appointment.**
- **Progressive weakness:** foot drop, meaning an inability to lift the front of your foot, or worsening leg weakness
- **Severe, unrelenting pain:** pain that does not improve with rest or with your usual measures
- **Fever with back pain:** may indicate infection
- **History of cancer:** new onset back pain, particularly with unexplained weight loss
- **Recent significant trauma:** a fall or car accident, especially if you are over 50 or have osteoporosis
- **Unexplained weight loss** accompanying back pain

Contact your healthcare provider promptly if:

- Symptoms worsen after 2 weeks of conservative care
- Pain is severe enough to prevent sleep
- You develop new areas of numbness or tingling
- Symptoms persist beyond 6 to 8 weeks without improvement

13. Building Your Personal Recovery Plan

Use this framework as a starting point for a conversation with your healthcare team, not as a substitute for one. Timelines are typical, not guaranteed, and your plan should be adjusted based on how you actually respond.

Weeks 1 and 2: Calm Things Down (Acute Phase)

The goal in weeks 1 and 2 is to calm things down. Discuss any supplement additions with your provider first. For movement, stick to short walks, prone on elbows, and gentle nerve glides if tolerated. Therapies at this stage include heat or ice for comfort, temporary modification of aggravating activities, and an initial evaluation by a physical therapist, chiropractor, or physician. On the diet side, begin the anti-inflammatory pattern and keep hydration steady, and for lifestyle, prioritize sleep, reduce stress load, and adjust your workstation.

Weeks 3 to 6: Rebuild (Subacute Phase)

Weeks 3 to 6 are about rebuilding. Review with your provider what supplements are helping and what are not. For movement, progress your directional exercises, add core stabilization, and increase walking duration. Therapies at this stage typically mean regular physical therapy or chiropractic care, with consideration of a trial of acupuncture or massage. Diet-wise, aim for the full anti-inflammatory pattern, and for lifestyle, work on building a repeatable stress management routine while keeping sleep hygiene consistent.

Weeks 7 to 12: Strengthen and Prevent (Rehabilitation Phase)

Weeks 7 to 12 focus on strengthening and prevention. Reassess the need for ongoing supplement use with your provider. Movement progresses to advanced core work, functional movement training, and a gradual return to normal activities. Therapies taper to as-needed care as you transition to a home exercise program. Diet becomes anti-inflammatory eating as a lifestyle, and for lifestyle, focus on maintaining your weight, keeping the exercise routine, and managing stress.

Beyond 12 Weeks: Maintenance

Beyond 12 weeks, the focus shifts to maintenance: continue core exercises several times per week, maintain the anti-inflammatory dietary pattern, and keep walking or another low-impact activity in the routine. Schedule periodic check-ins with your healthcare team, and address flare-ups early with your established toolkit.

If you are not improving on this timeline, that is information, not failure. Go back to your provider for re-evaluation.⁴⁹

Final Thoughts

Sciatica can be frightening and debilitating, but most cases improve significantly with conservative care. Your body has a real capacity to heal when given the right support: sensible nutrition, targeted movement, stress management, sleep, and patience.

Healing is rarely linear. You may have good days and bad days. The key is consistency with your plan and open communication with your healthcare team. If progress stalls, seek re-evaluation. Every person's sciatica is different, and your treatment should be tailored to your specific situation.

You are not alone in this, and with the right tools and support, meaningful improvement is a realistic goal.

References

Every reference below was retrieved and verified against the PubMed record. The PMID is provided so you can look up any of them yourself.

1. Konstantinou K, Dunn KM. Sciatica: review of epidemiological studies and prevalence estimates. *Spine (Phila Pa 1976)*. 2008;33(22):2464-2472. PMID 18923325.
2. Koes BW, van Tulder MW, Peul WC. Diagnosis and treatment of sciatica. *BMJ*. 2007;334(7607):1313-1317. PMID 17585160.
3. Chou R, Qaseem A, Snow V, et al. Diagnosis and treatment of low back pain: a joint clinical practice guideline from the American College of Physicians and the American Pain Society. *Ann Intern Med*. 2007;147(7):478-491. PMID 17909209. (Erratum: *Ann Intern Med*. 2008;148(3):247-248. PMID 18257154. Screened 2026-09-05; corrected, not retracted.)
4. Bush K, Cowan N, Katz DE, Gishen P. The natural history of sciatica associated with disc pathology. A prospective study with clinical and independent radiologic follow-up. *Spine (Phila Pa 1976)*. 1992;17(10):1205-1212. PMID 1440010.
5. Jacobs WC, van Tulder M, Arts M, et al. Surgery versus conservative management of sciatica due to a lumbar herniated disc: a systematic review. *Eur Spine J*. 2011;20(4):513-522. PMID 20949289.
6. Peul WC, van Houwelingen HC, van den Hout WB, et al. Surgery versus prolonged conservative treatment for sciatica. *N Engl J Med*. 2007;356(22):2245-2256. PMID 17538084.
7. Weinstein JN, Tosteson TD, Lurie JD, et al. Surgical vs nonoperative treatment for lumbar disk herniation: the Spine Patient Outcomes Research Trial (SPORT): a randomized trial. *JAMA*. 2006;296(20):2441-2450. PMID 17119140.
8. Atlas SJ, Keller RB, Wu YA, Deyo RA, Singer DE. Long-term outcomes of surgical and nonsurgical management of sciatica secondary to a lumbar disc herniation: 10 year results from the Maine Lumbar Spine Study. *Spine (Phila Pa 1976)*. 2005;30(8):927-935. PMID 15834338.
9. Calder PC. Omega-3 fatty acids and inflammatory processes: from molecules to man. *Biochem Soc Trans*. 2017;45(5):1105-1115. PMID 28900017.
10. Daily JW, Yang M, Park S. Efficacy of turmeric extracts and curcumin for alleviating the symptoms of joint arthritis: a systematic review and meta-analysis of randomized clinical trials. *J Med Food*. 2016;19(8):717-729. PMID 27533649.
11. Kuptniratsaikul V, Dajpratham P, Taechaarpornkul W, et al. Efficacy and safety of *Curcuma domestica* extracts compared with ibuprofen in patients with knee osteoarthritis: a multicenter study. *Clin Interv Aging*. 2014;9:451-458. PMID 24672232.
12. Plotnikoff GA, Quigley JM. Prevalence of severe hypovitaminosis D in patients with persistent, nonspecific musculoskeletal pain. *Mayo Clin Proc*. 2003;78(12):1463-1470. PMID 14661675.
13. Yamine K, Wehbe R, Assi C. A systematic review on the efficacy of vitamin D supplementation on diabetic peripheral neuropathy. *Clin Nutr*. 2020;39(10):2970-2974. PMID 32089370.
14. Shin HJ, Na HS, Do SH. Magnesium and pain. *Nutrients*. 2020;12(8). PMID 32718032.
15. Sun Y, Lai MS, Lu CJ. Effectiveness of vitamin B12 on diabetic neuropathy: systematic review of clinical controlled trials. *Acta Neurol Taiwan*. 2005;14(2):48-54. PMID 16008162.
16. Ametov AS, Barinov A, Dyck PJ, et al. The sensory symptoms of diabetic polyneuropathy are improved with alpha-lipoic acid: the SYDNEY trial. *Diabetes Care*. 2003;26(3):770-776. PMID 12610036. (Erratum: *Diabetes Care*. 2003;26(7):2227. Screened 2026-09-05; corrected, not retracted.)
17. Ziegler D, Ametov A, Barinov A, et al. Oral treatment with alpha-lipoic acid improves symptomatic diabetic polyneuropathy: the SYDNEY 2 trial. *Diabetes Care*. 2006;29(11):2365-2370. PMID 17065669.

18. Ziegler D, Nowak H, Kempler P, Vargha P, Low PA. Treatment of symptomatic diabetic polyneuropathy with the antioxidant alpha-lipoic acid: a meta-analysis. *Diabet Med*. 2004;21(2):114-121. PMID 14984445.
19. Paladini A, Fusco M, Cenacchi T, et al. Palmitoylethanolamide, a special food for medical purposes, in the treatment of chronic pain: a pooled data meta-analysis. *Pain Physician*. 2016;19(2):11-24. PMID 26815246.
20. Lang-Illievich K, Klivinyi C, Lasser C, et al. Palmitoylethanolamide in the treatment of chronic pain: a systematic review and meta-analysis of double-blind randomized controlled trials. *Nutrients*. 2023;15(6). PMID 36986081.
21. Yu G, Xiang W, Zhang T, et al. Effectiveness of Boswellia and Boswellia extract for osteoarthritis patients: a systematic review and meta-analysis. *BMC Complement Med Ther*. 2020;20(1):225. PMID 32680575.
22. Black CD, Herring MP, Hurley DJ, O'Connor PJ. Ginger (*Zingiber officinale*) reduces muscle pain caused by eccentric exercise. *J Pain*. 2010;11(9):894-903. PMID 20418184.
23. Wilson PB. Ginger (*Zingiber officinale*) as an analgesic and ergogenic aid in sport: a systemic review. *J Strength Cond Res*. 2015;29(10):2980-2995. PMID 26200194.
24. Crowley DC, Lau FC, Sharma P, et al. Safety and efficacy of undenatured type II collagen in the treatment of osteoarthritis of the knee: a clinical trial. *Int J Med Sci*. 2009;6(6):312-321. PMID 19847319.
25. Lam OT, Strenger DM, Chan-Fee M, Pham PT, Preuss RA, Robbins SM. Effectiveness of the McKenzie method of mechanical diagnosis and therapy for treating low back pain: literature review with meta-analysis. *J Orthop Sports Phys Ther*. 2018;48(6):476-490. PMID 29602304.
26. Peacock M, Douglas S, Nair P. Neural mobilization in low back and radicular pain: a systematic review. *J Man Manip Ther*. 2023;31(1):4-12. PMID 35583521.
27. McGill SM. Low back stability: from formal description to issues for performance and rehabilitation. *Exerc Sport Sci Rev*. 2001;29(1):26-31. PMID 11210443.
28. Searle A, Spink M, Ho A, Chuter V. Exercise interventions for the treatment of chronic low back pain: a systematic review and meta-analysis of randomised controlled trials. *Clin Rehabil*. 2015;29(12):1155-1167. PMID 25681408.
29. Vanti C, Andreatta S, Borghi S, Guccione AA, Pillastrini P, Bertozzi L. The effectiveness of walking versus exercise on pain and function in chronic low back pain: a systematic review and meta-analysis of randomized trials. *Disabil Rehabil*. 2019;41(6):622-632. PMID 29207885.
30. Geneen LJ, Moore RA, Clarke C, Martin D, Colvin LA, Smith BH. Physical activity and exercise for chronic pain in adults: an overview of Cochrane Reviews. *Cochrane Database Syst Rev*. 2017;4(4):CD011279. PMID 28436583.
31. Hahne AJ, Ford JJ, McMeeken JM. Conservative management of lumbar disc herniation with associated radiculopathy: a systematic review. *Spine (Phila Pa 1976)*. 2010;35(11):E488-E504. PMID 20421859.
32. Qaseem A, Wilt TJ, McLean RM, Forciea MA. Noninvasive treatments for acute, subacute, and chronic low back pain: a clinical practice guideline from the American College of Physicians. *Ann Intern Med*. 2017;166(7):514-530. PMID 28192789.
33. Paige NM, Miake-Lye IM, Booth MS, et al. Association of spinal manipulative therapy with clinical benefit and harm for acute low back pain: systematic review and meta-analysis. *JAMA*. 2017;317(14):1451-1460. PMID 28399251. (Errata: *JAMA*. 2017;317(21):2239, an error in 2 figures, PMID 28586870; and *JAMA*. 2017;318(20):2048, incorrect data in text, PMID 29183047. Screened 2026-09-05; corrected, not retracted. The correction text is subscription-gated and was not read. This guide reproduces no numeric value from this paper, so no figure here depends on it.)
34. Lewis RA, Williams NH, Sutton AJ, et al. Comparative clinical effectiveness of management strategies for sciatica: systematic review and network meta-analyses. *Spine J*. 2015;15(6):1461-1477. PMID 24412033.
35. Vickers AJ, Vertosick EA, Lewith G, et al. Acupuncture for chronic pain: update of an individual patient data meta-analysis. *J Pain*. 2018;19(5):455-474. PMID 29198932.

36. Han KH, Cho KH, Han C, et al. The effectiveness and safety of acupuncture treatment on sciatica: a systematic review and meta-analysis. *Complement Ther Med*. 2022;71:102872. PMID 35985442.
37. Cherkin DC, Sherman KJ, Kahn J, et al. A comparison of the effects of 2 types of massage and usual care on chronic low back pain: a randomized, controlled trial. *Ann Intern Med*. 2011;155(1):1-9. PMID 21727288.
38. French SD, Cameron M, Walker BF, Reggars JW, Esterman AJ. Superficial heat or cold for low back pain. *Cochrane Database Syst Rev*. 2006;(1):CD004750. PMID 16437495.
39. Johnson MI, Paley CA, Howe TE, Sluka KA. Transcutaneous electrical nerve stimulation for acute pain. *Cochrane Database Syst Rev*. 2015;(6):CD006142. PMID 26075732.
40. Manchikanti L, Nampiaparampil DE, Manchikanti KN, et al. Comparison of the efficacy of saline, local anesthetics, and steroids in epidural and facet joint injections for the management of spinal pain: a systematic review of randomized controlled trials. *Surg Neurol Int*. 2015;6(Suppl 4):S194-S235. PMID 26005584.
41. Friedly JL, Comstock BA, Turner JA, et al. A randomized trial of epidural glucocorticoid injections for spinal stenosis. *N Engl J Med*. 2014;371(1):11-21. PMID 24988555. (Erratum: *N Engl J Med*. 2014;371(4):390. Screened 2026-09-05; corrected, not retracted.)
42. Krause AJ, Prather AA, Wager TD, Lindquist MA, Walker MP. The pain of sleep loss: a brain characterization in humans. *J Neurosci*. 2019;39(12):2291-2300. PMID 30692228.
43. Finan PH, Goodin BR, Smith MT. The association of sleep and pain: an update and a path forward. *J Pain*. 2013;14(12):1539-1552. PMID 24290442.
44. Cherkin DC, Sherman KJ, Balderson BH, et al. Effect of mindfulness-based stress reduction vs cognitive behavioral therapy or usual care on back pain and functional limitations in adults with chronic low back pain: a randomized clinical trial. *JAMA*. 2016;315(12):1240-1249. PMID 27002445.
45. Saper RB, Lemaster C, Delitto A, et al. Yoga, physical therapy, or education for chronic low back pain: a randomized noninferiority trial. *Ann Intern Med*. 2017;167(2):85-94. PMID 28631003.
46. Shiri R, Karppinen J, Leino-Arjas P, Solovieva S, Viikari-Juntura E. The association between obesity and low back pain: a meta-analysis. *Am J Epidemiol*. 2010;171(2):135-154. PMID 20007994.
47. Shiri R, Lallukka T, Karppinen J, Viikari-Juntura E. Obesity as a risk factor for sciatica: a meta-analysis. *Am J Epidemiol*. 2014;179(8):929-937. PMID 24569641.
48. Chen LH, Weber K, Mehrabkhani S, Baskaran S, Abbass T, Macedo LG. The effectiveness of weight loss programs for low back pain: a systematic review. *BMC Musculoskelet Disord*. 2022;23(1):488. PMID 35606809.
49. Luijsterburg PA, Verhagen AP, Ostelo RW, et al. Effectiveness of conservative treatments for the lumbosacral radicular syndrome: a systematic review. *Eur Spine J*. 2007;16(7):881-899. PMID 17415595.
50. Germini F, Coerezza A, Andreinetti L, et al. N-of-1 Randomized Trials of Ultra-Micronized Palmitoylethanolamide in Older Patients with Chronic Pain. *Drugs Aging*. 2017;34(12):941-952. PMID: 29210011
51. Lugo JP, Saiyed ZM, Lane NE. Efficacy and tolerability of an undenatured type II collagen supplement in modulating knee osteoarthritis symptoms: a multicenter randomized, double-blind, placebo-controlled study. *Nutr J*. 2016;15:14. PMID: 26822714
52. Altman RD, Marcussen KC. Effects of a ginger extract on knee pain in patients with osteoarthritis. *Arthritis Rheum*. 2001;44(11):2531-2538. PMID: 11710709
53. Maroon JC, Bost JW. Omega-3 fatty acids (fish oil) as an anti-inflammatory: an alternative to nonsteroidal anti-inflammatory drugs for discogenic pain. *Surg Neurol*. 2006;65(4):326-331. PMID: 16531187

This guide was compiled from peer-reviewed medical literature and clinical best practices. It is intended to give you knowledge and options to discuss with your healthcare provider. It does not promise a cure and it is not a substitute for individualized medical care.

Working with Root Health

If your leg symptoms are not improving, or you are being asked to decide about surgery and want the conservative case laid out honestly first, I offer a free Root Discovery Session at Root Health to talk through where you are and what a reasonable next step looks like. You can schedule one at rootcauseunlocked.com/discovery, or reach the office at (919) 662-0520.

Consultations and ongoing care are both available wherever you are located. Being outside North Carolina is not a barrier to working with me. Tell me where you are when you reach out and I will lay out exactly how we would work together from there.

If you would rather start on your own, the free Root Cause Unlocked tracker has a mode built for musculoskeletal recovery, at rootcauseunlocked.com/tracker. Eight weeks of it turns “I think it might be helping” into something you and I can both read.

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